



Armada Kalamunda Group

Community Home Visit Safety Guideline (Orchard Avenue Centre and CRHTT)

1. Purpose

Armada Mental Health Service (AMHS) has a duty of care to provide optimal support and risk management strategies for staff working within consumers' homes or within community settings.

This guideline is to be applied in conjunction with the [East Metropolitan Health Service \(EMHS\) Community Home Visiting Policy](#).

2. Clinical staff procedure Orchard Avenue Centre (OAC) (business hours)

- Procedure is to be followed by all community Orchard Avenue Centre (OAC) clinicians during business hours.

All clinical staff are to complete a Community Home Visit Log (located in OAC MS Teams channel) prior to leaving the workplace. Designated OAC clerical staff will review the log.

Area	Designated Staff	Contact number
Orchard Avenue Centre (OAC)	Clerical Staff Team Leaders	93986600

A risk assessment is to be conducted prior to each home visit reviewing all the known factors, including consumer, environmental, PSOLIS alerts, and Risk Assessment Management Plan (RAMP). Consideration should be given to the following before conducting any community visit:

- Level of Risk for Police Attendance, and number of clinicians required to attend– refer to [EMHS Community/home visiting Policy](#) - page 3 for risk factors.
- Clinician gender allocation requirements based on any alerts or other sources of information. – refer [EMHS Chaperone Policy](#)
- All clinical staff are to ensure that the contact number for the OAC (**93986600**) is stored in the mobile they are using, and mobile phones are always fully charged and switched-on during home visits.
- Staff must only use government vehicles when home visiting. Prior to commencing a home visit, staff are to check that the vehicle has sufficient fuel, is not damaged, and is safe to drive.
- Community home visits should be recorded in the chronological order that they are to be undertaken in the community home visit log.
- Staff undertaking community home visits are to complete a safety check by voice call to the OAC every two hours on the hour after the commencement of the initial appointment.

- If a clinician is aware that they will be delayed in a particular place, clerical staff should be notified via telephone so the community home visit log can be amended.
- All clinical staff must ensure that they have an operational community duress alarm before going out on a home visit. These are to be tested before each home visit, with status of unit recorded on the community home visiting log.
- Any clinician in an emergency whilst in the community should activate the community duress alarm by pressing the SOS button.
- During business hours, if a clinician is aware, they will be unable to return to the workplace before 16:30:
 - They are to contact OAC clerical staff before 16:30 who will notify the Team Leader (or other senior clinician if Team Leader unavailable) and Leschen clerical reception staff.
 - OAC Clerical staff will update the home visiting log and provide Leschen clerical reception staff with the details required.
 - Leschen clerical reception staff will take responsibility for safety monitoring.
 - The clinician will inform Leschen clerical reception staff on their return to the workplace and the clinician will update the community home visit log.
- If clerical staff note that a clinician has not returned by 16:30, and has not made contact with OAC via their mobile:
 - OAC Clerical staff are to contact Leschen clerical reception staff and provide details required as per the log sheet.
 - Leschen clerical reception staff will wait 15 minutes and then attempt to contact clinician on their mobile phone, then:
 - After a further 5 minutes Leschen clerical reception staff will again attempt to contact clinician on their mobile phone.
 - If clinician cannot be contacted A/H CNS will assume the role of Urgent Response Coordinator (URC).
 - The clinician will inform the A/H CNS on their return to the workplace and update the community home visit log.

3. Clinical staff procedure OAC (after hours)

Procedure is to be followed by all community OAC ATT clinicians outside of OAC business hours (Monday to Friday after 16:30; Saturday, Sunday, and Public Holidays 08:00 to 20:00).

Mon – Sun 2000-2200 After Hours CNS is notified of all home visits for monitoring which is outside of staffed hours for Leschen clerical reception staff.

All clinical staff are to complete a community home visit log before leaving the workplace. Leschen clerical reception staff and A/H CNS will review the community home visit log.

The relevant section of the community home visit log is to be emailed to:

AHS.MHAferhoursCNS@health.wa.gov.au and,

AHS.LeschenReception@health.wa.gov.au

A risk assessment is to be conducted prior to each home visit reviewing all the known factors, including consumer, environmental, PSOLIS alerts, and RAMP. Consideration should be given to the following before conducting any community visit:

- Level of Risk for Police Attendance, and number of clinicians required to attend – refer to [EMHS Community/home visiting Policy](#) - page 3 for risk factors.
- All after hours home visits are a minimum of two persons.
- Clinician gender allocation requirements based on any alerts or other sources of information. – refer [EMHS Chaperone Policy](#)
- All clinical staff are to ensure that the contact numbers for the ATT After-Hours mobile phone (**0409125074**); and A/H CNS mobile phone (**0400 673 895**) are stored in the mobile they are using, and mobile phones are always fully charged and switched on whilst on home visits.
- MH A/H CNS is to be contacted by phone when community home visit log has been emailed. It is important to ensure the A/H CNS is notified as their mobile phone will be alerted in the event of a duress activation, and they also assume the role of **Urgent Response Coordinator (URC)** in the event of clinicians not responding to a scheduled safety check. If unable to contact A/H CNS by phone, Leschen clerical reception is to be contacted, and requested to bring this email to the attention of A/H CNS when available.
- The After-Hours ATT mobile is to remain in OAC for use of the 3rd clinician rostered on shift. This is to ensure that A/H CNS can contact OAC staff on site.
- Staff must only use government vehicles when home visiting.
- Community home visits should be recorded in the chronological order that they are to be undertaken in the community home visit log.
- Staff undertaking community home visits are to complete a safety check by voice call to the ATT After-hours mobile (when a 3rd clinician is rostered on shift) and Leschen clerical reception staff every two hours on the hour after the commencement of the initial appointment.
- When safety check telephone calls are received, Leschen clerical reception staff will complete their section of the community home visit log by ticking the relevant safety check box.
- If clinicians are aware that they will be delayed in a particular place, Leschen clerical reception staff should be notified via telephone so the community home visiting log can be amended.
- On return to base, staff are to contact Leschen clerical reception staff.
- All clinical staff must ensure that they have an operational community duress alarm before going out on a home visit. These are to be tested before each home visit, with status of unit recorded on the community home visit log.
- Any clinician in an emergency whilst in the community should activate the community duress alarm by pressing the SOS button.
- In the event of a safety check telephone call not being made by clinicians, Leschen clerical reception staff will wait 15 minutes and then attempt to contact the clinicians on their mobile phones, then:
- After a further 5 minutes Leschen clerical reception staff will again attempt to contact the clinicians on their mobile phones.
- If clinicians are unable to be contacted the A/H CNS will follow the (URC) procedure.

4. Duress pendant test procedure

Procedure to follow before every home visit:

- Activate the duress alarm as in a real situation.
- Advise the call centre when they answer, “this is a test” and to stand down. Clinicians will need to provide their ‘He’ number as per the false alarm process.
- Advise the call centre if a second alarm is going to be tested (two-person home visit).
- If a duress alarm is found to be faulty, remove from service and inform a Team leader.
- Update test outcome as ‘pass’ or ‘fail’ on Community Home visit log.

5. Clerical staff procedure OAC (business hours)

- The designated clerical staff member will be responsible for monitoring the community home visit log and responding to all incoming calls from clinicians conducting home visits.
- If the Clerical Coordinator Mental Health is unable to allocate clerical staff to this role, Team Leaders of ATT/CTT at OAC are to be notified to facilitate this role being undertaken.
- When safety check telephone calls are received, clerical staff will complete their section of the community home visit log by ticking the relevant safety check box.
- In the event of a safety check telephone call not being made by a clinician, clerical staff will wait 15 minutes and then attempt to contact the clinician on their mobile phone, then:
- After a further 5 minutes clerical staff will again attempt to contact the clinician on their mobile phone.
- If the clinician is unable to be contacted, clerical staff will initially attempt to contact the first person for their area nominated in the list below. If they are not available, contact with any of the named clinicians, as available, should be made:
 - **Adult Community (ATT, CTT, OA):** Team Leader for ATT or CTT in the first instance, then Program Manager Community Mental Health.
 - **Adult Community (ATT, CTT, OA)** Clinical Nurse Specialist if Team Leader(s) and Program Manager unavailable.
- The contacted staff member becomes the identified (URC).
- Clerical staff will continue to attempt contact on the clinician’s work mobile and immediately inform the (URC) if contact is made.

6. Clinical Staff operational procedure (CRHTT)

Area	Designated Staff	Contact number
CRHTT – 0800-1600	Ward Clerk	93912962
Leschen Mon – Fri 1600-2000 & Sat – Sun 0800 – 2000.	Leschen clerical team	93912300 & AHS.LeschenReception@health.wa.gov.au
Mental Health 2000-2200 Mon - Sun	After Hours CNS	040 067 3895 AHS.MHAfterhoursCNS@health.wa.gov.au

- All clinicians are provided with education on the use of the duress pendant, using the fleet car, completion of vehicle log sheet, safe parking of fleet cars during home visit, and ensuring fleet cars are fully stocked before the visit.
- CRHTT clinicians are to only travel in pairs. Prior to leaving the CRHTT office, the home visit log sheet must be completed with the following information: patient's address and phone number, NOK number, fleet car registration details, duress alarm unit number, attending clinicians' names and mobile numbers, time of arrival and departure. A copy of the home visit log form is kept by the CRHTT ward clerk.
- All clinicians complete the home visit safety tool assessment during suitability assessment. The tool encapsulates any identifiable risks, to assist clinicians to attend daily home visits for the period of admission to CRHTT.
- The CRHTT ward clerk during office hours and Leschen reception are responsible for monitoring and contacting the CRHTT clinicians. Mon – Sun 2000-2200 After Hours CNS is notified of home visits for monitoring. Clinicians must call and update their location after every visit, or at the estimated departure time listed on the home visit log sheet should they be running late. Safety measures are in place for clerical staff to call clinicians should they not call back at the dedicated time, if unable to contact clinicians on a visit, the second clinician's mobile number is tried, if still no answer, then NUM is notified during business hours and after 1600 the A/H CNS. The home visit log is to be emailed to Leschen reception at 1600 on AHS.LeschenReception@health.wa.gov.au.
- The Duress pendants are part of the risk mitigation procedure for CHRTT clinicians to utilise on daily home visits. They are to be worn on the clinicians in a manner to afford quick and easy access, and to enable clear communication with the duress pendant call centre. Duress signal strength is checked prior to entering the house. Clinicians are also aware that car keys and CRHTT mobile are always within reach during the home visit. Activation of the duress pendant will initiate a designated follow-up contact by the monitoring centre. The response may state either a confirmed SOS and process to be followed or a stand down due to an accidental activation.
- IOTAGG GPS Personal Duress poster and action plan placed in equipment room, for ease of access along with charging station for all pendants.
- Monthly duress testing process completed on arrangement with Iottag. The SOS system is monitored 24 hrs, 7 days a week.

7. Urgent Response Coordinator (URC) procedure

Upon being notified that a clinician's safety check has not been made, the (URC) is to take the following steps:

- Review the community home Visit log and telephone the clinician, if no response, telephone the consumer that was scheduled during the time frame. The (URC) will enquire if the staff member was present for the visit and what time they left.
- If the consumer advises that the staff member has left their premises, the (URC) will contact the next person on the community home visit log and request the same information.
- In the event that the clinician has not been located, the (URC) will contact the police on 131 444 and inform them of the situation.
- The (URC) will provide the police with the required information as per the community home visit log. This will include names, addresses, contact details, car registration and description and relevant history of all consumers. This information can be faxed as required.

- The (URC) will inform the Service Director, Head of Department, Program Manager and Nurse Coordinator Mental Health of the incident at the earliest convenience and inform the A/H CNS if out of business hours.
- If an emergency situation has occurred with risk exposure, complete Datix Clinical Incident Management System (CIMS) (consumer involvement only) and/or a Combined Hazard or Incident Reporting (CholR) Online Form and inform the line manager.
- Debriefing will occur as appropriate and will be coordinated by the Program Manager or Nurse Coordinator Mental Health.

Bushfire season considerations:

During the bushfire season 1 Dec–31 March it is the shared responsibility of clinical staff and the Urgent Response Coordinator (URC) to access up-to-date bushfire warnings and fire danger rating information before clinicians commence community visits. If a catastrophic fire danger rating is forecast for the area in which the visit is to take place, the visit is to be cancelled.

Clinical staff and URC are:

To monitor the emergency broadcasts to be aware of conditions affecting their relevant area via the [Emergency WA](#) website:

- Alerts and warnings – Details of current fires and any warnings that may have been issued
- Fire danger ratings – Details of areas rated severe, extreme or catastrophic
- Total fire bans – Areas are listed by affected Local Governments where a total fire ban has been declared.

[Bureau of Meteorology Warnings](#) website

- Weather warnings issued by the Bureau of Meteorology.

8. Legislative context

The following documents are required to give effect to this procedure:

- *Work Health and Safety Act 2020*
- *Mental Health Act 2014*
- *Public Health Act 2016*
- *Public Health Regulations 2017*
- Health Department of Western Australia (HDWA) [Code of Conduct \(MP 0031/16\)](#)
- East Metropolitan Health Service (EMHS) [Community/Home Visiting \(EMHS 131\)](#)
- EMHS [Prevention and Management of Workplace Aggression and Violence Policy \(EMHS 27\)](#)
- EMHS [Occupational Safety and Health Policy \(EMHS 123\)](#)
- EMHS [OSH: Incident and Hazard Reporting Policy \(EMHS 06\)](#)

9. Supporting information

The following documents support the implementation of this policy:

- Armadale Kalamunda Group [Bushfire Emergency Response Sub-Plan](#)
- [Duress Alarm Systems Policy](#)
- [Code Black Response – Orchard Avenue Centre Procedure.](#)

10. Compliance, monitoring and evaluation

Compliance against this guideline will be evaluated and monitored by the Mental Health Safety and Quality departmental meeting.

Monitored activities will include:

- Reported clinical incidents via Datix CIMS
- Reported incidents via CholR
- Quality audit of community home visiting log sheet.

11. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 1 - Action 1.30: The health service organisation:
- Standard 5 – Action 5.10: Clinicians use relevant screening processes:

National Standards for Mental Health Services (2010):

- Standard 2 – Criteria 2.6: The MHS meets their legal occupational health and safety obligations to provide a safe workplace and environment.
- Standard 2 – Criteria 2.12: The MHS conducts regular reviews of safety in all MHS settings, including an environmental appraisal for safety to minimise risk for consumers, carers, families, visitors and staff.

12. Glossary

The following definitions are relevant to this policy.

Term	Definition
Community Home Visit	Any clinical related activity/intervention conducted at any location other than: Armadale Mental Health Service, Eudoria Street Centre or Mead Centre.
Urgent Response Coordinator	A member of the identified group of senior staff responsible for initiating an urgent response.

13. Document owner

Shannon Stuart, Program Manager Community Program

Enquiries relating to this guideline may be directed to:

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Department: Mental Health
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14. Review

This guideline will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V4.0	24/02/2025	24/02/2028	Section 1: Inclusion of Orchard Avenue Centre within work area Section 2: Inclusion of Orchard Avenue centre to replace reference to Mead/Eudoria Section 3: CHRTT Procedure to be amended as stand alone

15. Authorisation

This guideline has been authorised and issued by the Service Director Mental Health.

Authorised by	Dene Olive – A/Service Director Mental Health
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